
DIAGNOSIS:

1) ACUTE GASTROENTERITIS WITH DEHYDRATION

2) URINARY TRACT INFECTION

HISTORY: C/O Multiple episodes of loose stools, 2-3 episodes of vomiting, fatigue since 3 days and fever since yesterday.

Initially she was treated at local clinic.

PAST HISTORY:K/C/O Thyroid disorder on treatment.

PHYSICAL EXAMINATION:

PR-89/min, BP-130/80 mmHg, RR-20/min, SPO2-98% at room air.

CNS-Conscious Oriented

CVS-S1S2(+)

RS-B/L NVBS(+)

PA-Soft, non tender

INVESTIGATIONS:

Reports Enclosed

COURSE IN THE HOSPITAL:

Patient presented to us with above mentioned complaints. After evaluation, she was admitted to ward. Initial investigations showed normal CBC. Serum creatinine (1.65mg/dl) done was elevated. Serum electrolytes done showed low serum sodium(128.00mmol/L). Urine routine done showed ketone bodies(+), 10-12/hpf of pus cells, 15-20/hpf of epithelial cells with presence of bacteria hence urine culture and sensitivity sent- report awaited. She was treated with IV fluids,

IV antibiotics, IV PPI's, IV antiemetics and other supportive measures. USG abdomen and pelvis done showed Grade-I fatty liver changes and mildly edematous part of ascending colon upto the hepatic flexure- could represent colitis. Repeat Serum Creatinine(1.17mg/dl) done was normal. TSH and Free T4 done were normal. Stool routine done showed 2-3/hpf of red blood cells, plenty/hpf of pus cells. Patient was adviced to stay back for further management but attenders not willing to the same, hence being discharged at request with following advice.

CONDITION AT DISCHARGE:

Hemodynamically stable

ADVICE ON DISCHARGE:

No.	Medication Name	Dosage	Frequency	Duration
1	TAB. RACIPER	40MG	1-0-0	7 DAYS (BEFORE FOOD)
2	TAB. EMESET	4MG	1-1-1	3 DAYS
3	TAB. OFLOX TZ		1-0-1	5 DAYS
4	TAB M STRONG		1-0-0	15 DAYS
5	TAB. ZEDOTT		1-1-1	3 DAYS
6	TAB. ENTRC		1-0-1	3 DAYS
7	TAB. MEFTAL SPAS		1 TAB SOS	4 TABLETS
8	TAB. LOPIRAMIDE	2MG	1-0-1	5 DAYS

FOLLOW-UP INSTRUCTIONS:

Urine culture and sensitivity sent- report awaited.

Review immediately in case of fever, loose stools, vomiting, fatigue.

Review on 09.03.2026. CBC

ER OBSERVATION CHART(1)

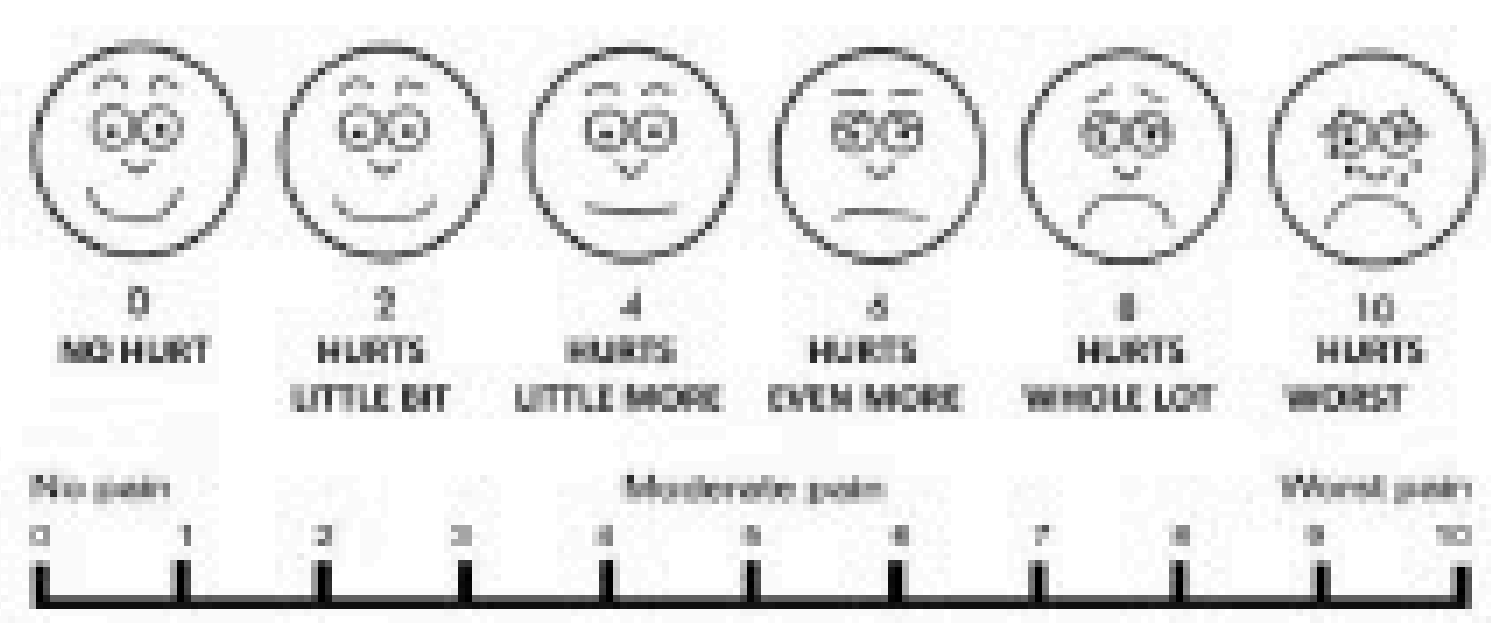
MRN:_____	Triage Category Red Yellow ☒ Green
ER ☒ OP ☐	MLC No-_____
Diagnosis: -DKA	Time of arrival ER Doctor: Time of response Time of response
	6:14pm 6:14pm

VITAL PARAMETERS

Time	6:14pm			Time	6:14pm		
Pulse	116b/m			Sao2 Oxygen			
BP	87/50			RR	22b/m		
Temp	98°F			Urine output (ml/hr)			
SaO2 on air	96%			Blood Glucose	443mg/dl		
GCS	15/5			Pain score	4/10		

Procedures _____ Pain scale 4/10

-> IV cannulization done in left hand 20g



Note: If score is > 2 please speak to consultant

Investigations: (Lab,Radiology,Pathology,others)	Time (patient /sample) sent	Time (patient / report) received	Referred To: Specialty Specialty Specialty Specialty Dr. Consultant	ie
CBL, creat, SE				
ABG, urine	Report	Report		
ECG	Sent to lab	Received		
			other orders:	

ER OBSERVATION CHART(2)							
IV Infusion	Doctor Sign	Amt	Rate	Verified by	Administered by	Start Time	End Time
INF NS	U	2 Bolus				6:14PM	9:00PM

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Medication Administration

Name of drug/ strength	Dosage & Route	Sign of Doctor	Time	Verified by	Administered by	Time
INF PAN	40mg / IV	U	6:14PM			6:14PM
INF EMESET	4mg / IV		6:14PM			6:14PM

- ☒ Patient advised admission to ward/ICU
 ☐ Patient advised discharge / referred to outside hospital
 ☐ Refusal of Treatment
- SDIU

INVESTIGATION CHECKLIST

[illegible]

NURSING DOCUMENTATION

(From.....8pm.....to.....8pm.....)

Date: 27/2/20

- 8:15 pm => Patient hand over taken from evening duty staff to night duty staff.
- > Patient Bed Rounds done
 - > Patient Conscious and Oriented
 - > Patient IV cannula present.
 - > Patient Foley's catheter present.
 - > Patient IV Fentanyl line on flow.
 - > Patient 2nd hourly GRBS monitoring present
 - > Patient vital signs monitored
Temp :- 98°F
Bp :- 110/54 mmHg
HR :- 85 bpm
RR :- 23 bpm
SpO₂ :- 98%
- 9pm -> Patient CT RUB plan, Foley's catheter clamped as per Dr. advice
- > Patient orally soft diet taken
- 10pm -> Patient had one episode of chills and Tachycardia, Tachypnea informed to Dr. and advice
Inj - Tramadol 2ml + 3ml NS = 5ml
3ml IV as per advice given.

NURSING DOCUMENTATION

(From.....8pm to.....8AM.....)

Date:

27/2/20

11pm → Patient CT KVB cancelled due to Night duty time due to patient's condition's

12pm → Patient had a fever spike Temp: 103°F informed to Dr. Rama ram and advise Inj - PCF 1gm as per advice given

1PM → Patient has settled

2PM → Patient had a Good Sleep.

5AM → Patient Bed bath given, Bed making done, comfortable position given.
→ Patient bowel opened.

@6AM Patient IV medication

Inj - Eureset 1mg, Inj - PAN 1mg,
Inj - Meropenem 1gm, Inj - HappyNerve plus given

@7AM Patient GRBS checked and recorded in chart.

@8AM Patient Hand over given to Morning duty staff

NURSING DOCUMENTATION

(From.....8 AM.....to.....2 PM.....)

Date: 28/2/20

- ⇒ patient change hand over
9 AM taken from night duty
Staff to monitor duty
Staff
- ⇒ patient side sounds is Done
- ⇒ patient vital signs check
and record
Bp - 120/80 mmHg
SpO₂ - 98%
HR - 65 bpm
RR - 20 bpm
- ⇒ patient IV Canner present (+)
- ⇒ patient Flap present (+)
- ⇒ patient to monitor
- ⇒ patient comfortable
position
- ⇒ patient CT xrd is Done
file is Done
- ⇒ patient IV fluid 150ml
per hour on going
- ⇒ patient seen by In
Chamber sig ADP to
Sig to word.
- ⇒ patient

NURSING DOCUMENTATION

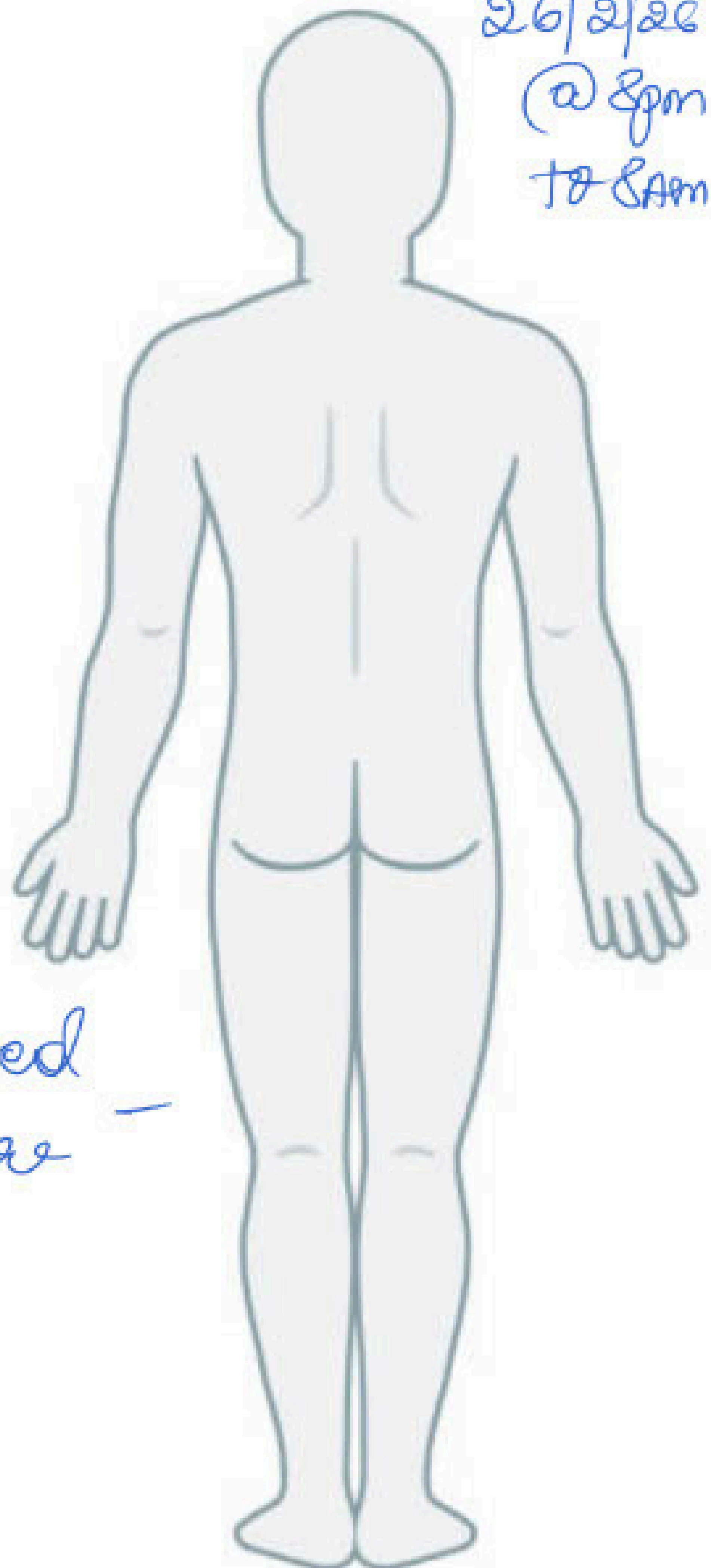
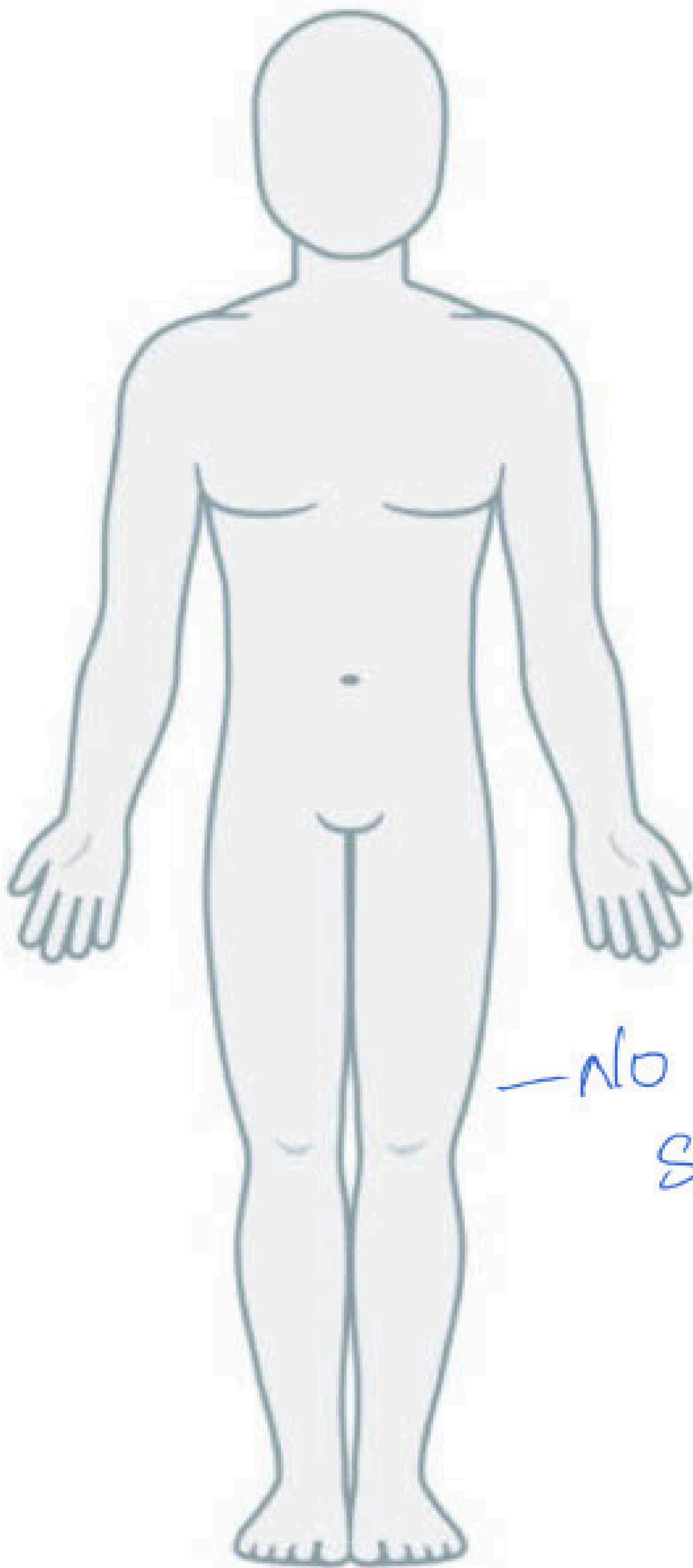
(From 8am to 2pm)

Date: 28/2/26

- 11AM => Patient Seen By Dr
Sir Adu
TO CBC/In (red) Th
=> patient slip to ward
wrap opening
=> Patient Seen By Dr
Sir Adu TO
C.S.T. (CT KUD super
=> patient slip to ward due
=> patient Pt- Carfax 10unit
given
=> patient IV medication
given as per drug chart
=> patient all super hand
over given to ward staff
=> patient LABS IS checked
and manual 2 hrs
2pm => pt chex, hand over given
to SREI staff to ward
staff

BED SORES

26/2/20
(@ 8pm
to 8am)



— No bed
Sore —

PULSES + -

Polyanka
(081)

Bed sores stage

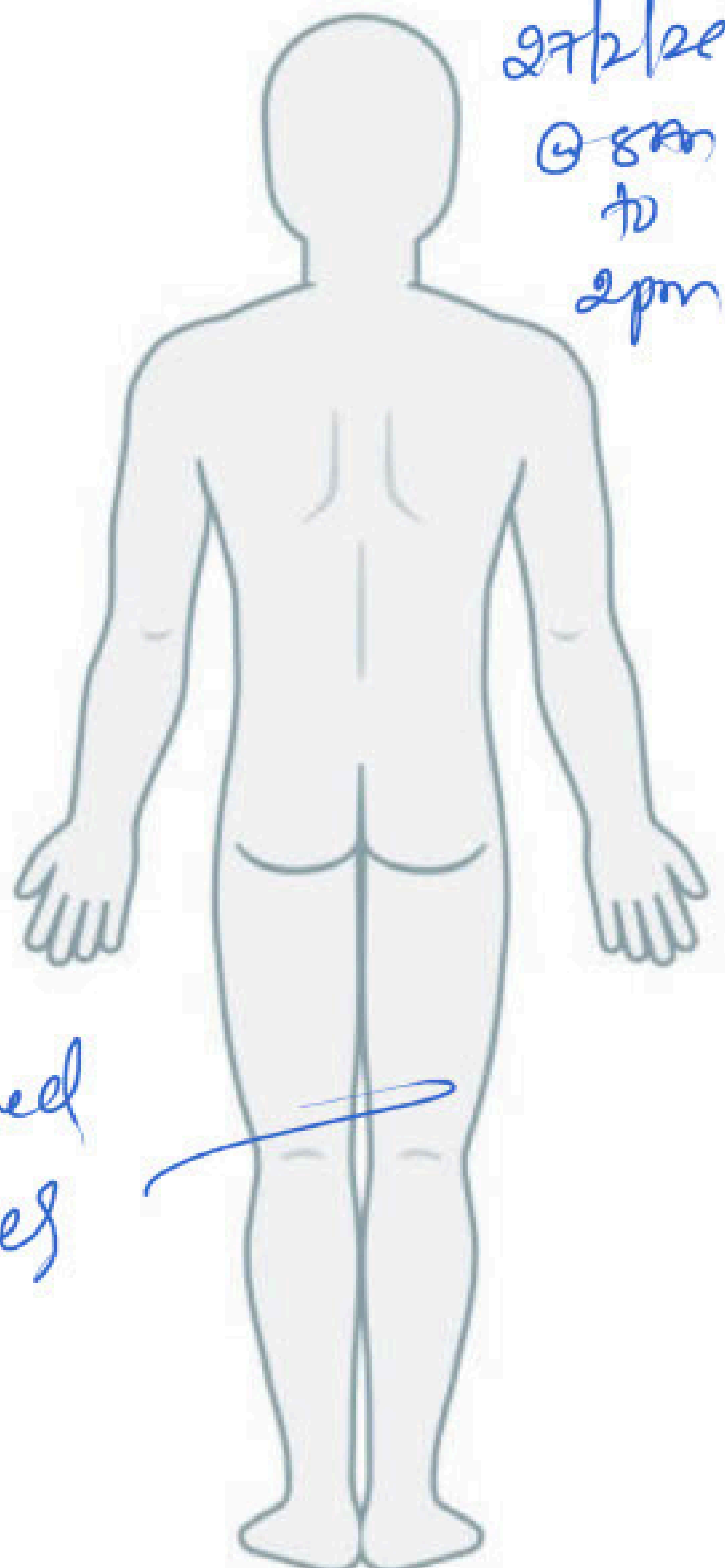
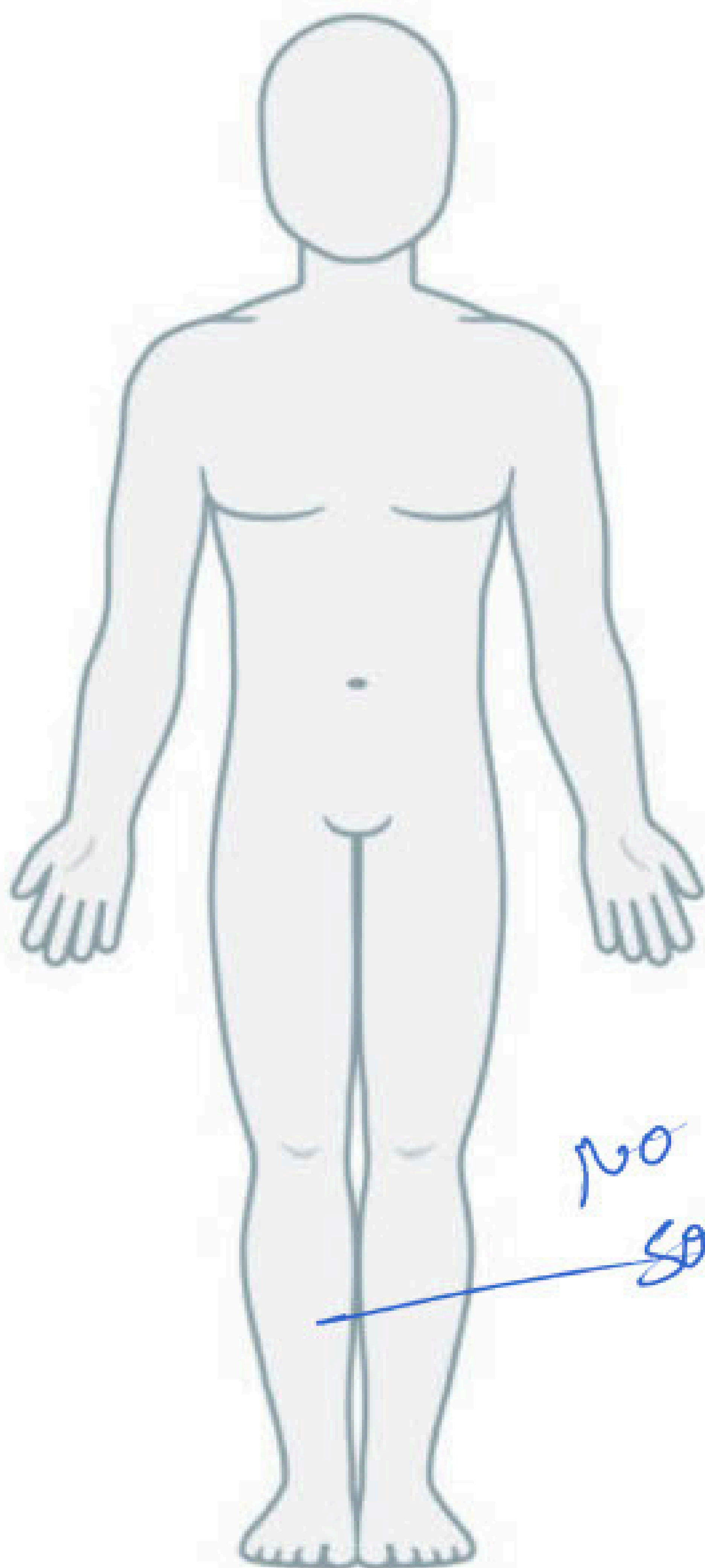
Stage 1. : Redness of the skin (upper

Stage 2. : Formation of blisters / Below surface of skin

Stage 3. : Damage to skin (second layer of skin into the fat issue

Stage 4. : Tissue Loss (Effect muscle & ligaments)

BED SORES



27/2/20
@ 8am
to
2pm

no bed
sores

PULSES + -

Bed sores stage

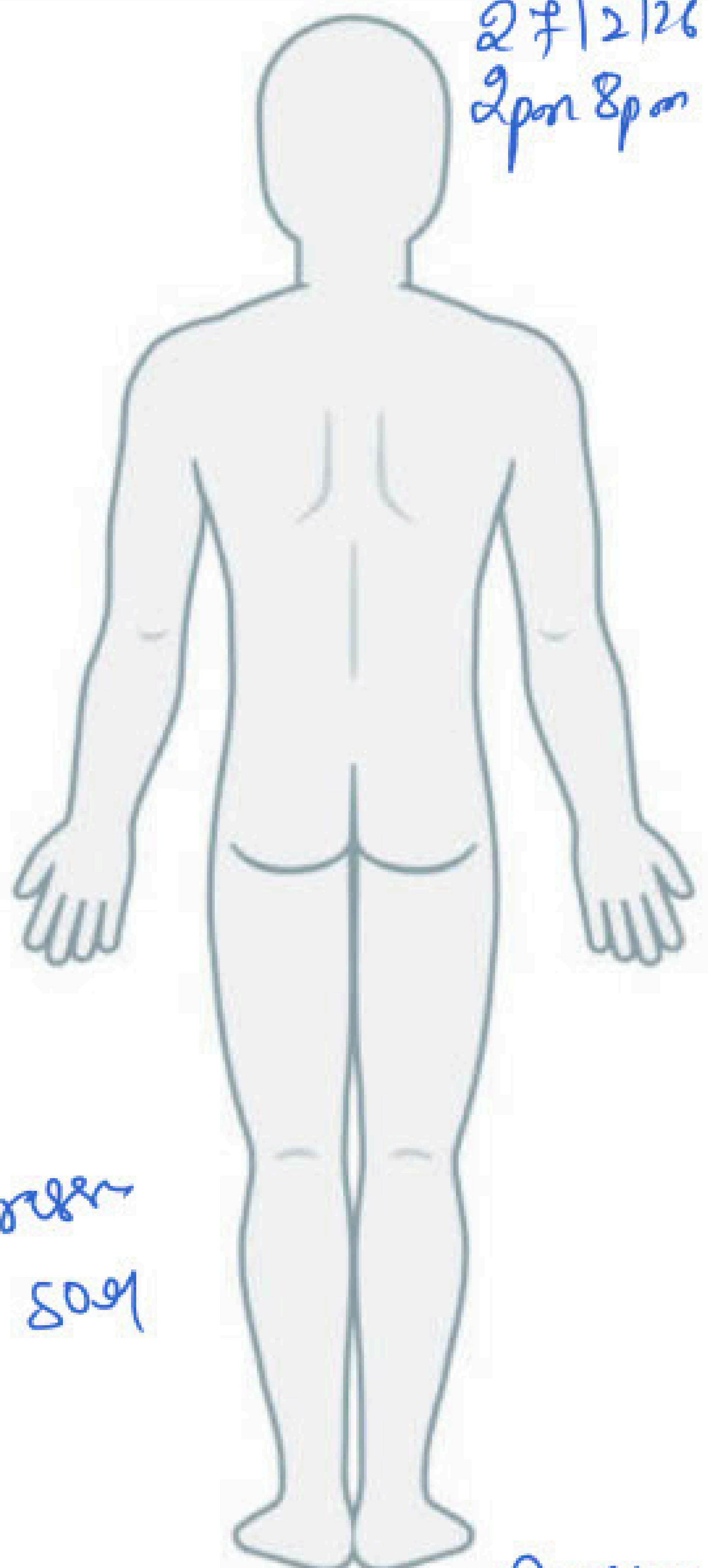
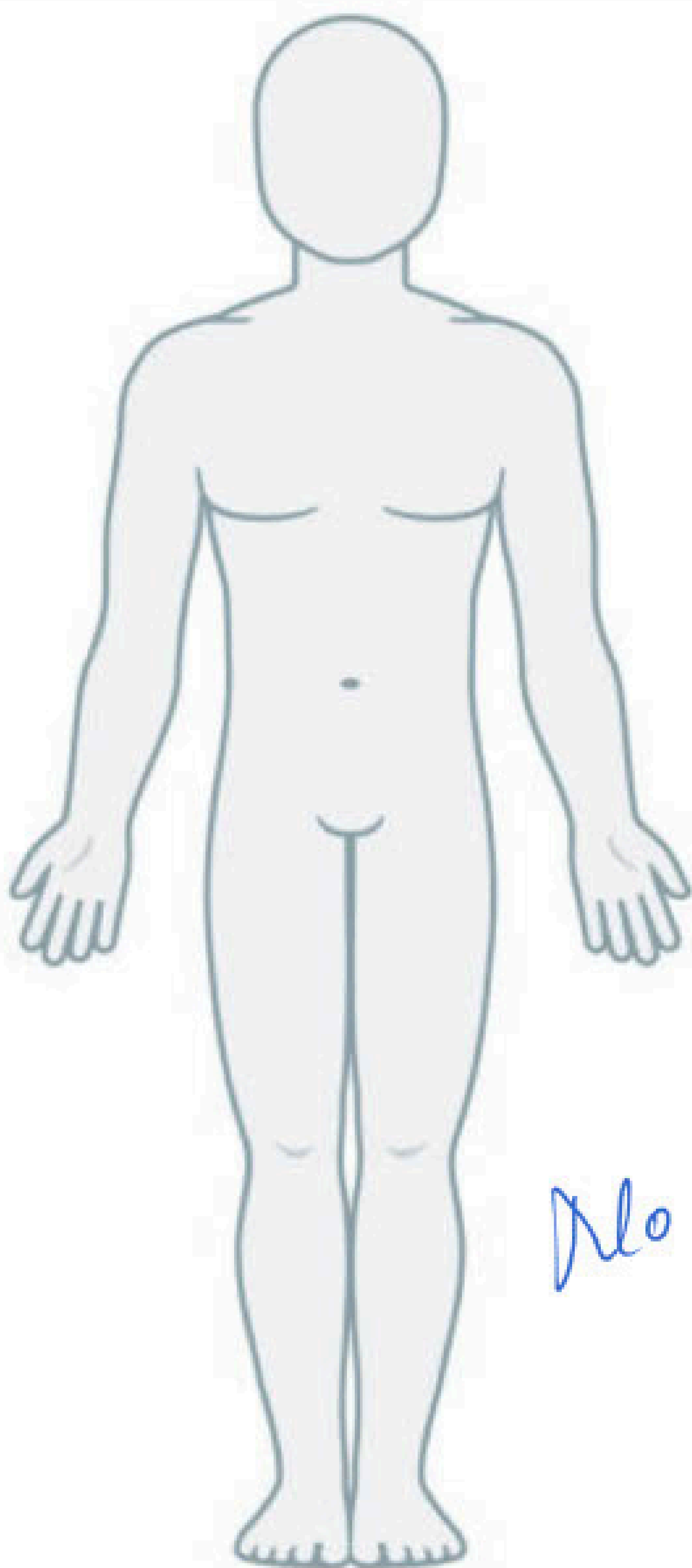
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Stage 2. : Formation of blisters / Below surface of skin

Stage 3. : Damage to skin (second layer of skin into the fat issue

Stage 4. : Tissue Loss (Effect muscle & ligaments)

BED SORES



27/2/26
2pm 8pm

No pressure
sore

Asht
0327

PULSES + -

Bed sores stage

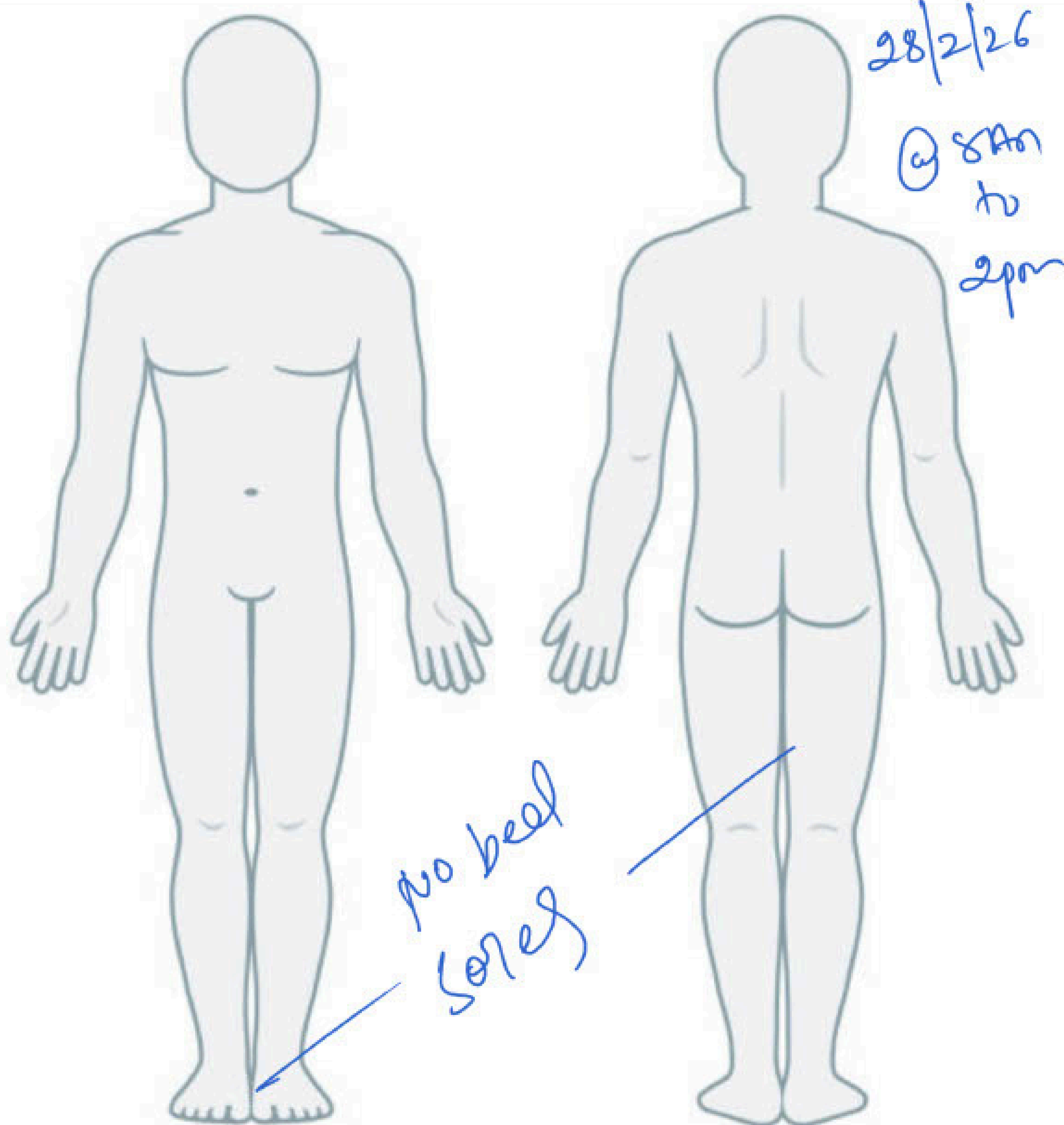
Stage 1. : Redness of the skin (upper

Stage 2. : Formation of blisters / Below surface of skin

Stage 3. : Damage to skin (second layer of skin into the fat issue

Stage 4. : Tissue Loss (Effect muscle & ligaments)

BED SORES



PULSES + -

Bed sores stage

Stage 1. : Redness of the skin (upper

Stage 2. : Formation of blisters / Below surface of skin

Stage 3. : Damage to skin (second layer of skin into the fat issue

Stage 4. : Tissue Loss (Effect muscle & ligaments)

PROCEDURE CHART			
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Weight: 71kg			Department: Internal medicine			
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Date	26/2/26	27/2/26	28/2/26	1/3/26	2/3/26			
Procedures								
IV Cannulization	yes (18-G)	yes 18G	yes 18G	yes (18G)	yes 21G			
N.G. Tube Inscertion								
Felley's Catheterisation	yes 16F	yes 16G	yes 16G	PK Rumour				
L.P.								
I.C.D.								
C.V.P.								
Intubation								
P.D.								
Phototherapy		yes						
Urine C./S.		yes						
Blood C./S.								
PD Sets & Vein O Line	yes (10cm)	yes	yes	yes	yes			
Oxygen	yes							
Ventilator								
Bubble CPAP								
NP02								
HFNC								
Viral Serology/Study								
Bone marrow								
USG/ECHO		yes						
Tapping								
Blood Transfussion								
Dressing			no	no	no			

Signature								
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NURSING ASSESSMENT ON ADMISSION				
Date & Time of Nursing Initial Assessment: 26-2-26 6:14 PM General Consent Signed: Yes / No				
Vital Signs	Temp 98°F	Pulse : 116 b /Min	Resp : 22 b /Min	BP : 87/50
Height	Weight 71kg	Blood Group	SPO2 (if required) 98	GRBS (if required) 443
Complaints during admission and its duration	Fever, generalized weakness since 3 days			
Neurological	Oriented: Yes / No		Speech : Clear / Slurred	Speech : Clear / Slurred
	Level of Consciousness Conscious / Semiconscious / Unconscious		Visual Impalment : None / Wears Spectacles / Blind-L R	Extremity Strength : Equal Unequal
	Pupils : Equal / Unequal / Reactive / Sluggish		Hearing Impairment : None Uses Aids / Deaf - L - R	Pain : Yes / No Location : Body
Pain Scale	 0 No Pain  2 Little Pain  4 Mild Pain  6 Moderate Pain  8 Severe Pain  10 Worst Pain			
Airway / Breathing	Airway : Clear / Obstructed Dyspnea : Present / Absent Wheezing : Yes / No		Cough : Yes / No Sputum : Yes / No	
Circulation	Edema : Present / Absent Location of Edema Skin Perfusion : Warm / Dry / Cool / Diaphoretic		Capillary refill : < 3 Secs / > 3 Secs	
Skin & Mucous Membrane	Colour : Fresh / Pale / Flushed / Cyanotic / Jaundiced Temp : Cool / Warm / Hot Moisture : Dry / Clammy		Turgor : Good / Fair / Poor Mucous membrane : Intact / Impaired Condition : Pink / Pale / Moist / Dry	
Gastrointestinal	Loss of Appetite : Yes / No Nausea / Vomiting / Diarrhoea Constipation		Abdomen : Soft / Firm / Tender / Distention / Expelling Flatus	Swallowing difficulty : Yes / No
Genitourinary	Dysuria : Yes / No Frequency : Incontinence : Yes / No		Hematuria : Yes / No	
Any Drug Allergy	NOT known			

NURSES NOTES

Known Drug Allergies - not known -

Date & Time Observation / Nsg Action / Response / Plan Staff Name & ID

26/2/26

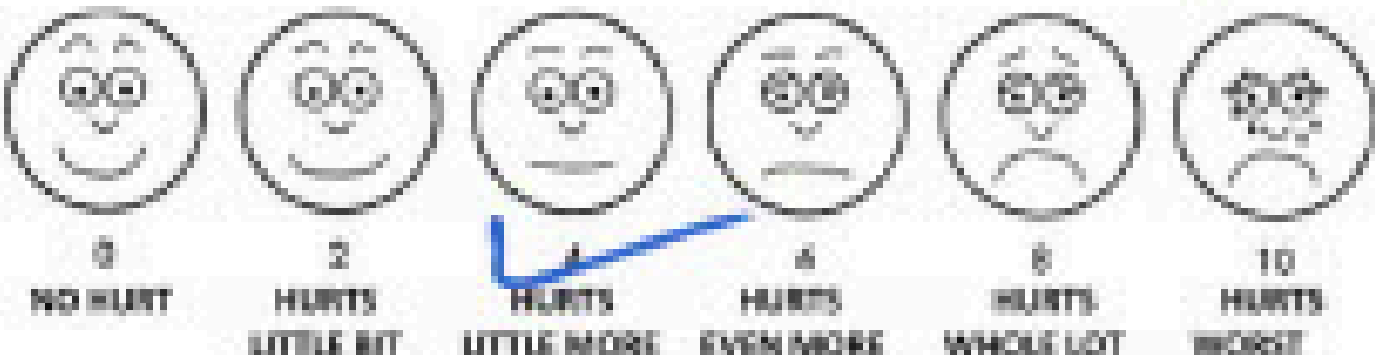
- patient Referred to casualty.
- patient complaints to fever and general weakness.
- patient conscious and oriented.
- patient checked the vital signs and recorded in ER chart.
- patient seen by Dr.
- patient IV cannulation done on R hand.
- patient IV medications given.
- patient blood investigations sent to lab collect the all Reports.
- patient ECG done.
- patient moved to bed position.
- patient case and blood reports inform to Dr. Shunya Sampad.
- patient Admission to HDU.
- patient Hand out to 3rd floor HDU evening staff.

NB : All injections to be initiated by persons administering and witnessing

* Date Of Admission

** Known Drug Allergies

26/2/26
not known



NURSES NOTES

Known Drug Allergies

not known

Date & Time

Observation / Nsg Action / Response / Plan

Staff Name & ID

28/2/26
up

patient received to EDU
to 1st floor line
patient said sounds don't
patient described &
conscious

EC cannula & ep band
parents PL parents

Dr.

5:30pm

EC sounds don't adjust
CT ERS 8th hourly
PL Remove TM
CBC ser. creat TM

8pm

patient changes heard
eyes given to right
duty shift

8pm

patient changes heard
our taken from evening
duty shift

patient conscious &
oriented

patient in cannula & ad
band present

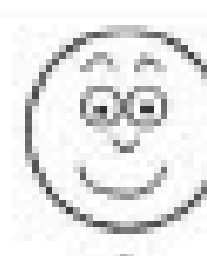
9pm

patient vital checked
& recorded to moni
-toring chart

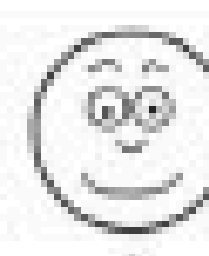
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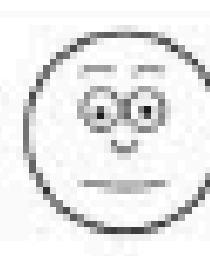
** Known Drug Allergies



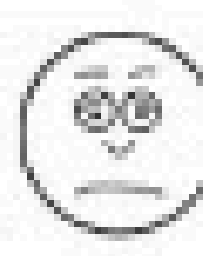
0
NO HURT



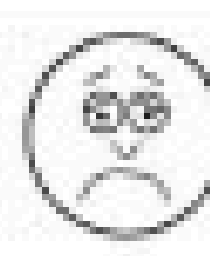
2
HURTS
LITTLE BIT



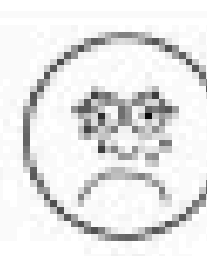
4
HURTS
LITTLE MORE



6
HURTS
EVEN MORE



8
HURTS
WHOLE LOT



10
HURTS
WORST

NURSES NOTES

Known Drug Allergies

- not known -

Date & Time

Observation / Nsg Action / Response / Plan

Staff Name & ID

10PM ->

patient oral medication
given as per drug
chart

11PM ->

patient temp 100°P to
inform MR Roberts
SR to advise CP

1/13/26

12PM ->

patient stillunch
don't no any fresh
complaint

2PM ->

patient stillunch
don't patient slept
well

3AM ->

patient sick bed
meeting done

6AM ->

patient in medical
given as per drug
chart

7AM ->

patient CBC, S count
send to lab report
due

7:15AM ->

patient PIC removed

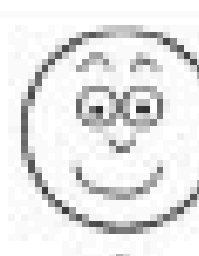
8AM ->

patient change hand
over given to morning due
to

NB : All injections to be initiated by persons administering and witnessing

* Date Of Admission

** Known Drug Allergies



0
NO HURT



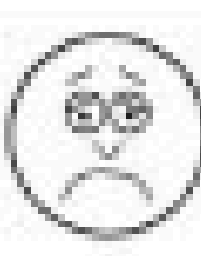
2
HURTS
LITTLE BIT



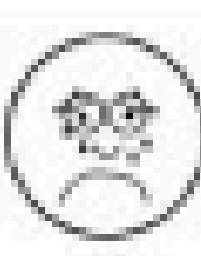
4
HURTS
LITTLE MORE



6
HURTS
EVEN MORE



8
HURTS
WHOLE LOT



10
HURTS
WORST

NURSES NOTES

Known Drug Allergies

not known

Date & Time

Observation / Nsg Action / Response / Plan

Staff Name & ID

13/26
9am

patient change hand over
taken to night duty staff
to morning duty staff
patient said wounds done
patient oriented & consi-

ous
in armole & Ep band
present
vital signs checked &
recorded the monitor
- sig check

10am oral medication advised
as per drug check

11:40am Dr. nurse seen
dent advise CBT

2pm oral medication given as
per drug check

4pm Patient vitals are checked &
recorded

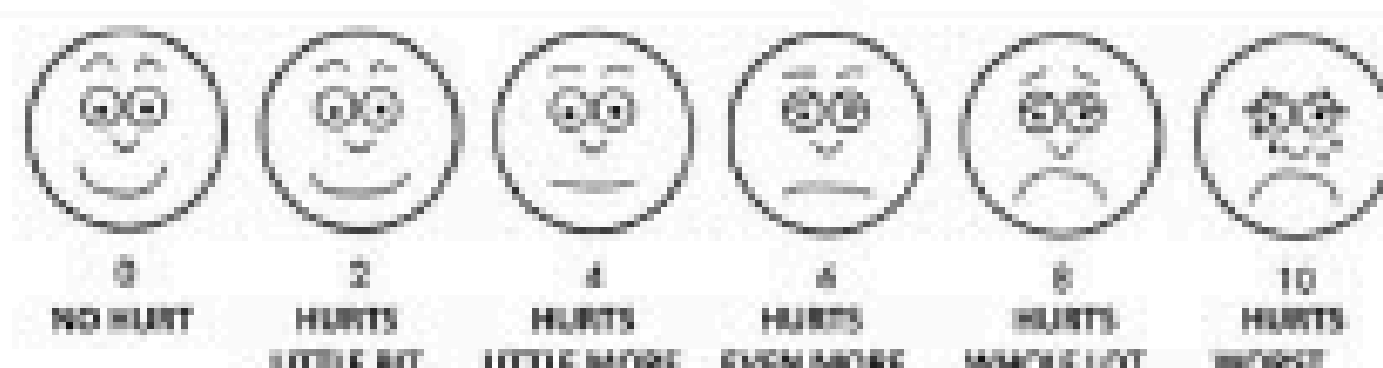
patient Temperature checked
Temperature is 99.1F Incom

6:45pm to Duty doctor and patient
spong bath given

NB : All injections to be initiated by persons administering and witnessing

* Date Of Admission

** Known Drug Allergies



NURSES NOTES

Known Drug Allergies

Date & Time

Observation / Nsg Action / Response / Plan

Staff Name & ID

7pm Patient Chief complaints on leg pain, Inform to duty doctor Advice No sumo

8pm 1pm 2pm Patient hand over given to night duty staff

8pm -> patient changed hand over taken from evening duty staff

-> patient conscious & oriented

-> patient in cannula & AD band present

9pm -> patient vitals checked & recorded to monitor -ing chest

10pm patient oral medication given as per drug

2/3/26 chest

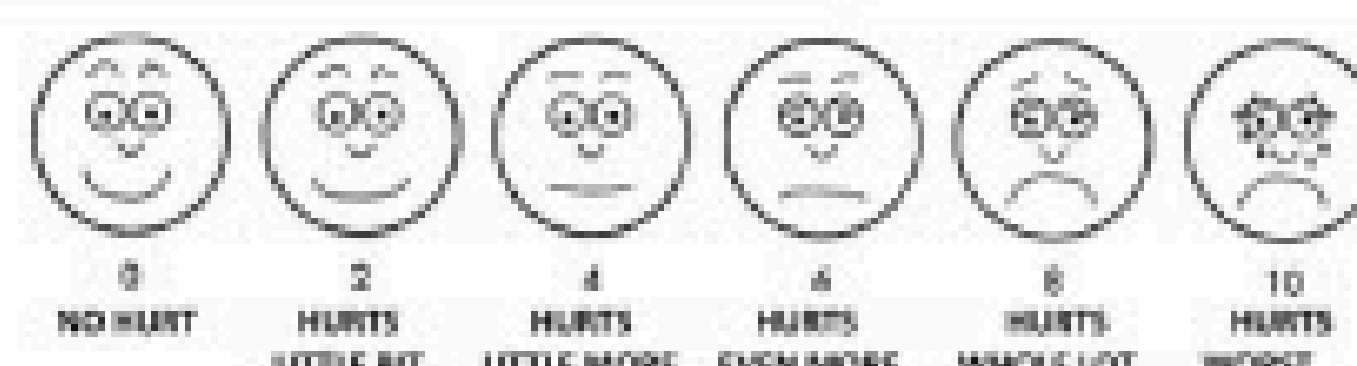
12pm patient side signs done. no any fresh complaints

2pm -> patient side signs done. patient slept well

NB : All injections to be initiated by persons administering and witnessing

* Date Of Admission

** Known Drug Allergies



NURSES NOTES

Known Drug Allergies

- not known -

Date & Time

Observation / Nsg Action / Response / Plan

Staff Name & ID

5AM

patient side bed meeking

done

6AM

patient on medication given as per drug chart

7AM

patient vitals checked & recorded to chart

8AM

patient charge hand on give to morning duty staff

8am

Patient hand over taken from Night duty staff

→ Patient is conscious & oriented

→ Patient vitals are checked & recorded

9am

→ Patient side sounds re done

10am

Patient oral medication advice as per drug chart

11:30am

patient seen by RN for advice discharge

on Request of ortho opinion

→

patient seen by RN

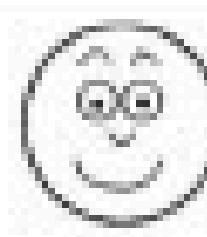
12pm

advice x-ray after received inform reports

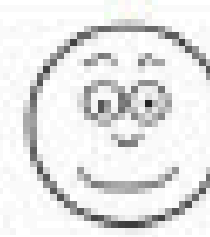
NB : All injections to be initiated by persons administering and witnessing

* Date Of Admission

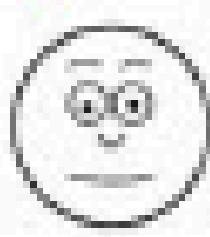
** Known Drug Allergies



0 NO HURT



2 HURTS LITTLE BIT



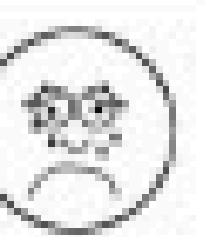
4 HURTS LITTLE MORE



6 HURTS EVEN MORE



8 HURTS WHOLE LOT



10 HURTS WORST

NURSES NOTES

Known Drug Allergies

Date & Time

Observation / Nsg Action / Response / Plan

Staff Name & ID

2PM Parent hr send to discharge
Summary & billing

→ file received to summary
& billing

2PM → Advise medication explain
health education given

→ OPD file original summary
all original reports
send to patient attorney

3PM → Ep band removed
Ev cannula not removed

→ Ev discharge began
Ev medication Advised

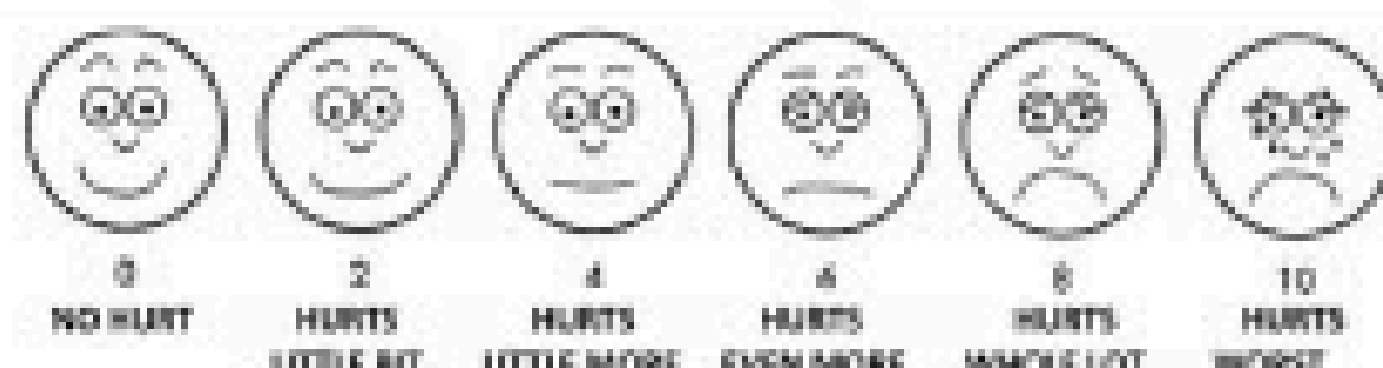
→ Billing clear, & clear
ship received

4PM → patient get discharge.

NB : All injections to be initiated by persons administering and witnessing

* Date Of Admission

** Known Drug Allergies



DISCHARGE CHECK LIST

SL NO	DETAILS	REMARKS
1	PATIENT DETAILS FULL NAME AGE/GENDER IP NO DATE OF ADMISSION DATE OF DISCHARGE	
2	DISCHARGE SUMMARY	lives ✓
3	DISCHARGE MEDICATION	Advised by ✓
4	BLOOD REPORTS	lives ✓
5	RADIOLOGY REPORTS-CT/X-RAY	lives ✓
6	USG REPORTS	lives ✓
7	MRI REPORTS	N/A
8	ECG	lives ✓
9	ECHO	lives ✓
10	PATIENT IP BANDS	Removed ✓
11	IV CANNULA	not removed ✓ Be doing
12	CVP	N/A
13	OXYGEN	N/A
14	OTHER REPORTS OR FORMS	lives ✓
15	OPD FILE AND OLD REPORTS	lives ✓
16	BILLING CLEARANCE	lives ✓
17	ICU Chart	attached ✓

DISCHARGE STAFFS NAME:
CROSS CHECKED INCHARGE
RECEIVED PATIENT OR ATTEND

DIABETS HOSPITAL MONITORING CHART

DATE	values	BBF	ABF	BL	AL	BD	AD	Midnight	Remarks
28/2/26	BS			267		389			2
	Insulin dose			16U		8U			
1/3/26	BS	265		178		270 mgdl			3
	Insulin dose	22U H.A		12IU (Ins Lantus)		12IU			
2/3/26	BS	217		167 mgdl					1
	Insulin dose	18U (H.A)		12IU (Ins Lantus)					
	BS								
	Insulin dose								
	BS								
	Insulin dose								
	BS								
	Insulin dose								
	BS								
	Insulin dose								

BS-Blood Sugar(GBSL/RBSL)

CAUTI(Catheter Associated Urinary Tract Infection)

DATE OF Insertion: 27/2/22 TIME: 2:30 PM inserted By Melvina

	Days	27/2/22			28/2/22			3			4			5			6			7		
		M	E	N	M	E	N	M	E	N	M	E	N	M	E	N	M	E	N	M	E	N
Check the clinical indication why the catheter is in-situ. is it still required		Yes	Yes	Yes	Yes	Yes	Yes															
Routine daily meatal hygiene (catheter care)?		Yes	Yes	Yes	Yes	Yes	Yes															
Perform hand hygiene and wear gloves prior to catheter care/insertion/maintenance?		Yes	Yes	Yes	Yes	Yes	Yes															
Indwelling catheters secured?		Yes	Yes	Yes	Yes	Yes	Yes															
Sterile, continuously closed drainage system used?		Yes	Yes	Yes	Yes	Yes	Yes															
Catheter and drainage tube always connected?		Yes	Yes	Yes	Yes	Yes	Yes															
Unobstructed urine flow present?		Yes	Yes	Yes	Yes	Yes	Yes															
Collecting bag regularly emptied?		Yes	Yes	Yes	Yes	Yes	Yes															
Collecting bag always below the level of the bladder?		Yes	Yes	Yes	Yes	Yes	Yes															

Signature of Staff Nurse

CAUTI(Catheter Associated Urinary Tract Infection)

INVESTIGATION	RESULT VALUE	UNIT	REFERENCE RANGE
CLINICAL PATHOLOGY REPORT			
URINE ROUTINE			
Physical Examination			
URINE PH	5.5		
COLOUR	PALE YELLOW		
APPEARANCE	SLIGHTLY TURBID		
SPECIFIC GRAVITY	1.015		
Chemical Examination			
URINE ALBUMIN	PRESENT +		
SUGAR	PRESENT 1.5%		
<u>KETONE BODIES</u>	<u>PRESENT +</u>		
BILE SALT	ABSENT		
BILE PIGMENT	ABSENT		
Microscopic Examination			
PUS CELLS	4-5/hpf		
EPITHELIAL CELLS	1-2/hpf		
RBC'S	2-3/hpf		
NITRITE	ABSENT		
BACTERIA	ABSENT		
Impression:	AMORPHOUS URATES PRESENT.		
End of Report			

Please correlate laboratory results clinically. Contact laboratory for any clarification. All results are sample specific.

INVESTIGATION	RESULT VALUE	UNIT	REFERENCE RANGE
BIOCHEMISTRY REPORT			
SERUM CREATININE	0.85	mg/dL	0.7 - 1.4
RANDOM BLOOD SUGAR (RBS)	<u>496.00</u>	mg/dl	<u>80 - 140</u>
SERUM ELECTROLYTES			
<u>S.SODIUM</u>	<u>127.00</u>	mmol/L	<u>135 - 150</u>
S.POTASSIUM(K+)	3.64	mmol/L	3.5 - 5
<u>S.CHLORIDE</u>	<u>84.90</u>	mmol/L	90 - 108
End of Report			

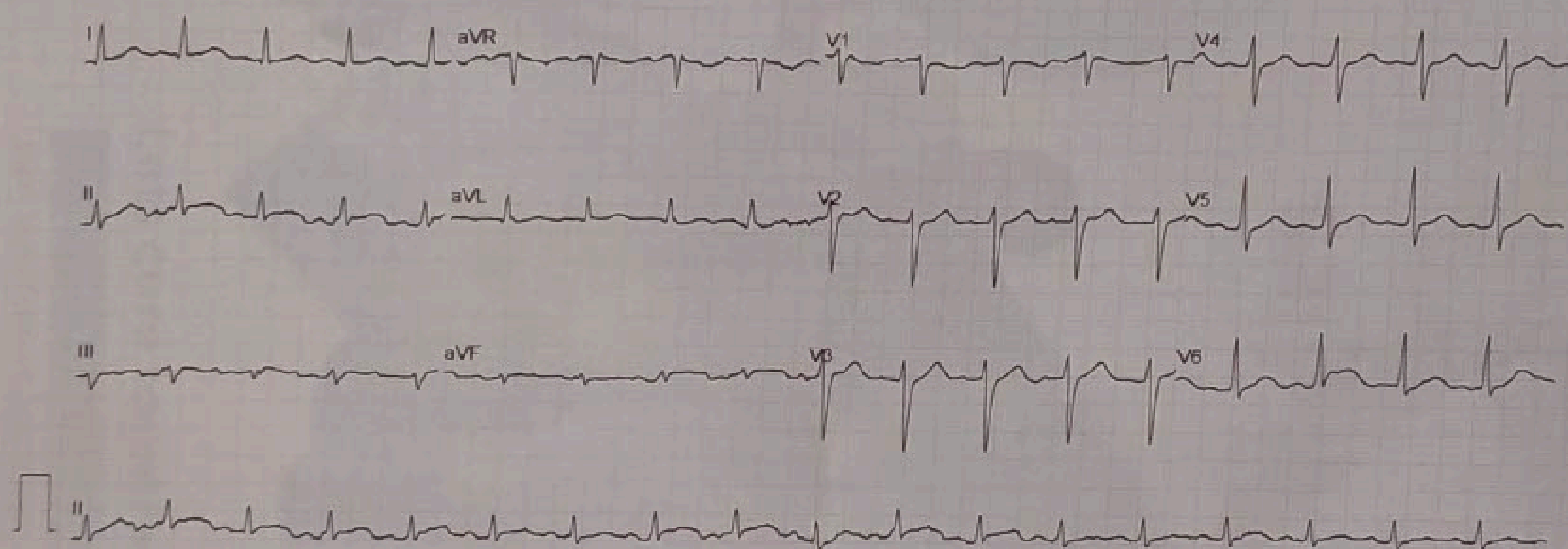
Please correlate laboratory results clinically. Contact laboratory for any clarification. All results are sample specific.

INVESTIGATION	RESULT VALUE	UNIT	REFERENCE RANGE
BIOCHEMISTRY REPORT			
ABG(Arterial Blood Gas) WITH LACTATE			
pH	<u>7.39</u>	mm Hg	7.35 - 7.45
pCO2	32.00	mm Hg	
pO2	72	mmHg	
Sodium [Na+]	<u>114</u>	mmol/L	136 - 146
Potassium [K]	<u>2.65</u>	mmol/L	3.4 - 4.5
Chloride [Cl-]	<u>83</u>	mmol/L	98 - 107
cCALCIUM	<u>0.71</u>	mmol/L	1.15 - 1.33
HCT	36	%	35 - 50
HAEMOGLOBIN	<u>11.80</u>	g/dL	12 - 16
SO2(C)	94.20	%	
CHCO3	<u>19.20</u>	mmol/L	20 - 28
BE	-5.80	mmol/L	
LACTATE	0.80	mmol/L	0.3 - 20
End of Report			
Please correlate laboratory results clinically. Contact laboratory for any clarification. All results are sample specific.			

INVESTIGATION	RESULT VALUE	UNIT	REFERENCE RANGE
HAEMATOLOGY REPORT			
COMPLETE BLOOD COUNT (CBC)			
HAEMOGLOBIN (HB)	<u>12.0</u>	gm/dl	13.5 - 17.5
TOTAL COUNT	6680	Cells/cumm	4000 - 11000
NEUTROPHILS	<u>79.1</u>	%	35 - 66
LYMPHOCYTES	<u>12.1</u>	%	24 - 44
MONOCYTES	<u>7.9</u>	%	3 - 6
EOSINOPHILS	0.6	%	0 - 6
BASOPHILS	0.3	%	0 - 1
PLATELET COUNT	1.80	Lakhs/cumm	1.5 - 4.5
RBC COUNT	4.57	mill/cumm	4.5 - 5.9
PACKED CELL VOLUME(PCV)	<u>35.9</u>	%	37 - 53
MCV	<u>78.6</u>	fL	80 - 100
MCH	26.3	pg	25 - 35
MCHC	33.4	g/dl	32 - 36
RDW-CV	12.2	%	11.5 - 14
MEAN PLATELET VOLUME(MPV)	10.9	fL	6.5 - 12
End of Report			
Please correlate laboratory results clinically. Contact laboratory for any clarification. All results are sample specific.			

TEST DESCRIPTION	RESULT	UOM	BIOLOGICAL REFERENCE RANGE
HAEMATOLOGY			
COMPLETE BLOOD COUNT			
Haemoglobin (Automated/Blood)	11.4	gm/dL	13.0 - 17.0
Total WBC Count (Automated/Blood)	7160	Cells/cmm	4000 - 11000
Differential Count (DC)			
Neutrophils	78	%	42 - 72
Lymphocytes	17	%	20 - 50
Eosinophils	1	%	00 - 06
Monocytes	4	%	02 - 10
Basophil	00	%	00 - 01
Packed Cell Volume - PCV (Automated/Blood)	32.8	%	35 - 48
RBC Count - (Red Blood Cell) (Automated/Blood)	4.25	Million/cmm	4.0 - 5.5
Mean Corpuscular Volume - MCV	77.2	fL	80.0 - 100.0
Mean Corpuscular Hemoglobin - MCH	26.8	pg	27.0 - 32.0
Mean Corpuscular Hemoglobin Concentration - MCHC	34.8	gm/dL	31.0 - 35.0
Platelet Count (Automated/Blood)	<u>2.56</u>	Lakhs/cmm	1.40 - 4.50
BIOCHEMISTRY			
Glycosylated Hemoglobin - HbA1c			

	RESULT	UOM	Reported On : NULL
Mean Blood Glucose	352.2	mg/dL	
SEROLOGY			
CRP - (C-Reactive Protein) (Immunoturbidimetry/Serum)	88.23	mg/L	
			New Born
			0 days :Less than 0.6 mg/l
			1 day :Less than 3.2 mg/l
			7 Days :Less than 1.6 mg/l
			Adults :Less than 5.0 mg/
Widal Test - Slide Method			
Salmonella typhi "O"	Positive 1:160 Dilutions		
Salmonella typhi "H"	Positive 1:80 Dilutions		
Salmonella paratyphi "AH"	Negative 1: 20 Dilutions		
Salmonella paratyphi "BH"	Negative 1: 20 Dilutions		
Note : 1. Titres 1:80 and above of "O" antigen & 1:80 and above of "H" antigen are significant			
2. Rising titres are significant			



GE MAC2000 1.1 12SL™ V241 25 mm/s 10 mm/mV ADS 0.56-20 Hz 50 Hz 4x2.5x3_25_R1 Unconfirmed 1/1

USG ABDOMEN & PELVIS (BEDSIDE)

- ◆ Liver (17cm) is enlarged in size and shows diffuse grade I fatty infiltration. No focal lesion detected to the extent imaged. No IHBR / EHBR dilatation. CBD is not dilated.
- ◆ Gall bladder is well distended and has normal wall thickness. Shows a conglomerated calculus measuring 13mm. No evidence of pericholecystic fat stranding at the time of scan.
- ◆ Spleen (11cm span) is normal in size and echotexture. No focal lesion detected to the extent imaged.
- ◆ Pancreas - Visualized part of pancreas is normal in size and echotexture.
- ◆ Both kidneys are mildly bulky in size. Normal in shape, position and echotexture. Corticomedullary differentiation is maintained. No focal mass detected to the extent imaged. Right kidney measures: 12.0cm. No dilatation of pelvicalyceal system in the present study. No calculi seen to the extent imaged. Left kidney measures: 12.3cm. No dilatation of pelvicalyceal system in the present study. No calculi seen to the extent imaged.
- ◆ HRUSG reveals, no significant mass formation / collection to the extent imaged.
- ◆ Urinary bladder is empty. Foley's bulb in situ.
- ◆ Prostate: Suboptimal.
- ◆ Minimal ascites. Minimal right pleural effusion noted with underlying subsegmental lung consolidation.

IMPRESSION:

- ☐ Mild hepatomegaly with grade I fatty infiltration.
- ☐ Cholelithiasis without cholecystitis.
- ☐ Mildly bulky bilateral kidneys. Suggested RFT correlation towards pyelonephritis.
- ☐ Minimal ascites.
- ☐ Minimal right pleural effusion with underlying subsegmental lung consolidation.

INVESTIGATION	RESULT VALUE	UNIT	REFERENCE RANGE
CLINICAL PATHOLOGY REPORT			
URINE ROUTINE			
Physical Examination			
URINE PH	5.5		
COLOUR	<u>YELLOW</u>		
APPEARANCE	<u>TURBID</u>		
SPECIFIC GRAVITY	1.025		
Chemical Examination			
URINE ALBUMIN	<u>PRESENT ++</u>		
SUGAR	<u>PRESENT 1.0%</u>		
KETONE BODIES	<u>PRESENT +++</u>		
BILE SALT	<u>PRESENT</u>		
BILE PIGMENT	<u>PRESENT</u>		
Microscopic Examination			
PUS CELLS	3-4/hpf		
EPITHELIAL CELLS	2-3/hpf		
RBC"S	2-3/hpf		
NITRITE	ABSENT		
BACTERIA	ABSENT		
Impression:	GRANULAR CAST PRESENT		
	End of Report		

Please correlate laboratory results clinically. Contact laboratory for any clarification. All results are sample specific.

ADULT TRANS-THORACIC ECHO REPORT

PATIENT NAME:

REF. DOCTOR :

6

FINDINGS

ATRIA

LEFT ATRIUM : Normal

RIGHT ATRIUM : Normal

ATRIOVENTRICULAR VALVES

MITRAL VALVE : Normal

TRICUSPID VALVE : Normal

VENTRICLES

LEFT VENTRICLE : Normal

RIGHT VENTRICLE : Normal

SEPTAE

INTERATRIAL SEPTUM : Intact

INTERVENTRICULAR SEPTUM : Intact

SEMILUNAR VALVES

AORTIC VALVE : Normal

PULMONARY VALVE : Normal

PERICARDIUM : No Pericardial Effusion

THROMBUS / VEGETATION : Nil

DOPPLER

VALVES	PEAK VELOCITY	PEAK GRADIENT	REMARKS
MITRAL	E&A-FUSED	-	
TRICUSPID	0.6 m/sec	TR Gradient -28mmHg	MR-Trivial
AORTIC	1.4m/sec	7 mmHg	TR- Mild
PULMONARY	0.8 m/sec	2 mmHg	AR-Trivial

M MODE

AO :22mm	LVIDd :38mm	IVSd :10mm	EDV : ml
LA :28mm	LVIDs : 25mm	PWd: 10mm	ESV : ml
	LVEF : 60%	FS : 30%	TAPSE: 18mm

OTHER: Sinus Tachycardia {HR-149bpm}
IVC-12/6 mm, Normal Collapsing

WALLMOTION ABNORMALITIES : NO DEFINITE RWMA

FINAL DIAGNOSIS :

- ❖ NO DEFINITE RWMA
- ❖ NORMAL LV SYSTOLIC FUNCTION
- ❖ LVEF-60%
- ❖ AR/MR-TRIVIAL
- ❖ TR-MILD
- ❖ NO CLOT / VEGETATION

INVESTIGATION	RESULT VALUE	UNIT	REFERENCE RANGE
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BIOCHEMISTRY REPORT

SERUM CREATININE	1.07	mg/dL	0.7 - 1.4
------------------	------	-------	-----------

End of Report

INVESTIGATION	RESULT VALUE	UNIT	REFERENCE RANGE
HAEMATOLOGY REPORT			
COMPLETE BLOOD COUNT (CBC)			
HAEMOGLOBIN (HB)	<u>10.4</u>	gm/dl	13.5 - 17.5
TOTAL COUNT	7820	Cells/cumm	4000 - 11000
NEUTROPHILS	<u>72.6</u>	%	35 - 68
LYMPHOCYTES	<u>13.0</u>	%	24 - 44
MONOCYTES	<u>10.9</u>	%	3 - 6
EOSINOPHILS	3.1	%	0 - 6
BASOPHILS	0.4	%	0 - 1
PLATELET COUNT	1.28	Lakhs/cumm	1.5 - 4.5
RBC COUNT	4.01	mill/cumm	4.5 - 5.9
PACKED CELL VOLUME(PCV)	<u>32.1</u>	%	37 - 53
MCV	80.0	f	80 - 100
MCH	25.9	pg	25 - 35
MCHC	32.4	g/dl	32 - 36
RDW-CV	12.9	%	11.5 - 14
MEAN PLATELET VOLUME(MPV)	10.6	fL	6.5 - 12
Impression:	MANUAL PLATELET COUNT DONE.		
End of Report			
Please correlate laboratory results clinically. Contact laboratory for any clarification. All results are sample specific.			

TARTERIES

Ascending Aorta : Normal
Aortic Arch : Normal
Pulmonary Artery : Normal
No
Cardium : No Pericardial Effusion
Thrombus / Vegetation : Nil

DOPPLER

VALVES	PEAK VELOCITY	PEAK GRADIENT	REMARKS
MITRAL	E&A-FUSED	-	MR-Trivial
AORTIC	0.6 m/sec	TR Gradient -28mmHg	TR- Mild
PULMONARY	1.4m/sec	7 mmHg	AR-Trivial
	0.8 m/sec	2 mmHg	

4 MODE

AO :22mm	LVIDd :38mm	IVSd :10mm	EDV : ml
LA :28mm	LVIDs : 25mm	PWd: 10mm	ESV : ml
	LVEF : 60%	FS : 30%	TAPSE: 18mm

OTHER: Sinus Tachycardia {HR-149bpm}
IVC-12/6 mm, Normal Collapsing

WALLMOTION ABNORMALITIES : NO DEFINITE RWMA

FINAL DIAGNOSIS :

- ❖ NO DEFINITE RWMA
- ❖ NORMAL LV SYSTOLIC FUNCTION
- ❖ LVEF-60%
- ❖ AR/MR-TRIVIAL
- ❖ TR-MILD
- ❖ NO CLOT / VEGETATION

DONE BY :Sourav Krishna J (Bsc.CCT)

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Blood Group:

Allergies: No Known Allergies. Hbar:

1040

100

NE

CONTRACT
100-100000

128

IT

1040 Unpublished T.M., Dr. Picheng

Time	Activity	Notes
10:00	Arrive	10:00
10:15	Start	10:15
10:30	Work	10:30
10:45	Work	10:45
11:00	Work	11:00
11:15	Work	11:15
11:30	Work	11:30
11:45	Work	11:45
12:00	Work	12:00
12:15	Work	12:15
12:30	Work	12:30
12:45	Work	12:45
13:00	Work	13:00
13:15	Work	13:15
13:30	Work	13:30
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23:30	Work	23:30
23:45	Work	23:45
24:00	Work	24:00

Investigation				Date of writing	Journal by
Title					
Object					
Method					
Result					
Conclusion					
References					
Appendix					
Bibliography					
Index					
Cover					
Spine					
Endpaper					
Title page					
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List of appendices					
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25th Edition		20th Edition		19th Edition	
Continuum	Yes				
Letters	2000-2001				
Books	100-200				
CDs	None				
8					
100	1000				
Number of	a	b	c	d	e
Percent of	100%	100%	100%	100%	100%

F	A	S	T	H	U	G	S	A	B	I	D
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ICU Care Plan:

Dr. A. C. C. C. C.

- After
- Follow the steps
- GRS mostly & lastly
- If meeting
- Welcome of you
- the study on the last
- In your list
- 2 steps to hand, one

Referral Address:

Patient Briefing Note:

Friday, 10/11/19

[illegible][illegible][illegible]

totally soft dul
ns/r (at 150 ml/hr)

ICU-CHART

SCORE

2: 26.02.26

Weight

POD:

Blood Group:

Allergies - No Known - Hboc

ECG:

EF:

VC:

Culture Report

BLOOD

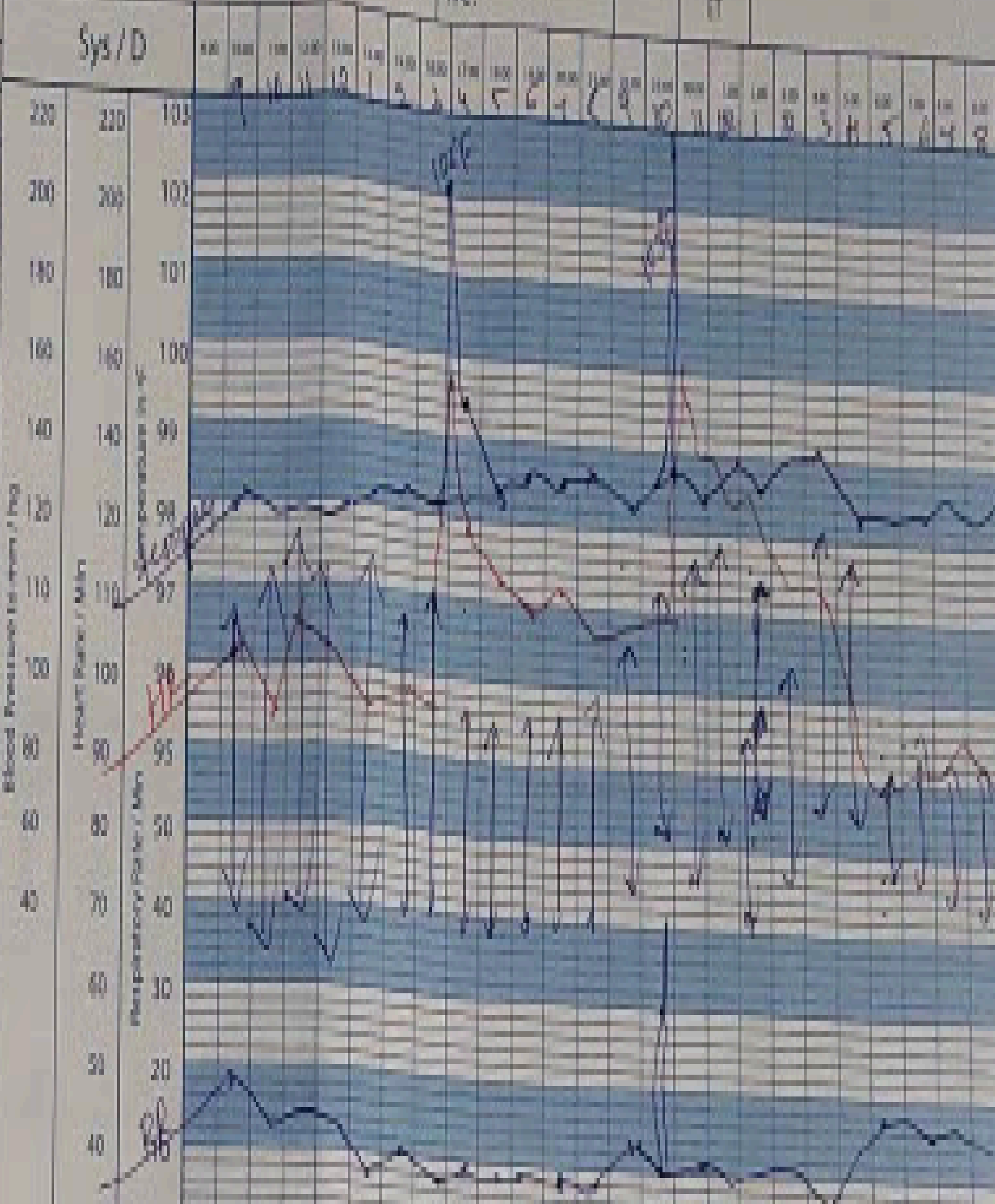
URINE

ET

Diagnosis: 1AF2

UNCONTROLLED T₃ DM

Drug	Dose	Route	Frequency	Start Date	End Date	Indication	Notes
INT. PARACETAMOL	40mg	IV	12hr	26/02/26			
INT. DMSO	4mg	IV	12hr	26/02/26			
1. INT. ZENAMAX-ET	1.5g	IV	12hr	26/02/26			
INT. SUMOL	1g	IV	12hr	26/02/26			
INT. HADY NERVE PLEX	1mg	IV	12hr	26/02/26			
T. DOLU	60mg	PO	12hr	26/02/26			
1. LACTOS	100	SC	00/12	26/02/26			
INT. HADY NERVE PLEX	1mg	IV	12hr	26/02/26			
INT. HADY NERVE PLEX	1mg	IV	12hr	26/02/26			



Parameter	00	04	08	12	16	20	24
Sys/D	100/60	100/60	100/60	100/60	100/60	100/60	100/60
HR	60	60	60	60	60	60	60
RR	12	12	12	12	12	12	12
Temp	36.0	36.0	36.0	36.0	36.0	36.0	36.0

Parameter	00	04	08	12	16	20	24
Sys/D	100/60	100/60	100/60	100/60	100/60	100/60	100/60
HR	60	60	60	60	60	60	60
RR	12	12	12	12	12	12	12
Temp	36.0	36.0	36.0	36.0	36.0	36.0	36.0

Parameter	00	04	08	12	16	20	24
Sys/D	100/60	100/60	100/60	100/60	100/60	100/60	100/60
HR	60	60	60	60	60	60	60
RR	12	12	12	12	12	12	12
Temp	36.0	36.0	36.0	36.0	36.0	36.0	36.0

Parameter	00	04	08	12	16	20	24
Sys/D	100/60	100/60	100/60	100/60	100/60	100/60	100/60
HR	60	60	60	60	60	60	60
RR	12	12	12	12	12	12	12
Temp	36.0	36.0	36.0	36.0	36.0	36.0	36.0

ICU Care Plan:
AB & Chandra
→ Oral diet
→ GCS mainly 5/6
→ Sp. mainly
→ Analgesic
→ 1 Shift to ward - by evening
→ Blood/Urine L/S

Referral Advice:
Patient Briefing Note:
- Only updates

Time	Date	Time of Day	Time of Night	Time of Morning	Time of Afternoon	Time of Evening	Time of Night
00	26/02/26	00	04	08	12	16	20
04	26/02/26	04	08	12	16	20	24
08	26/02/26	08	12	16	20	24	00
12	26/02/26	12	16	20	24	00	04
16	26/02/26	16	20	24	00	04	08
20	26/02/26	20	24	00	04	08	12
24	26/02/26	24	00	04	08	12	16

Time	Date	Time of Day	Time of Night	Time of Morning	Time of Afternoon	Time of Evening	Time of Night
00	26/02/26	00	04	08	12	16	20
04	26/02/26	04	08	12	16	20	24
08	26/02/26	08	12	16	20	24	00
12	26/02/26	12	16	20	24	00	04
16	26/02/26	16	20	24	00	04	08
20	26/02/26	20	24	00	04	08	12
24	26/02/26	24	00	04	08	12	16

26.02.2024

8pm POD:

Blood Group:

Allergies: - Not Known - Hbnc:

EF:

NC:

BLOOD

URINE

ST

Diagnosis: /AF/

(Uncontrolled) T. DM

INI PANTOAC

2nd EMERG

2nd ZOMAX-ES

2nd Simol

2nd HAPPY NERVE PLUS

T. DOLO

Sys/D

220	120	103
200	100	102
180	180	101
160	160	100
140	140	99
120	120	98
100	100	97
80	80	96
60	60	95
40	40	94
20	20	93
10	10	92

Temperature in °C

Heart Rate / min

Respiratory Rate / min

SpO2

ET

ECG

ECG

ECG

ECG

ECG

ECG

ECG

ECG

ECG

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ECG

Investigation				Copy of Questionnaire	Completed by
1	2	3	4	5	6
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1	2	3	4	5	

INVESTIGATION	RESULT VALUE	UNIT	REFERENCE RANGE
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BIOCHEMISTRY REPORT

SERUM CREATININE	1.04	mg/dL	0.7 - 1.4
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End of Report

Please correlate laboratory results clinically. Contact laboratory for any clarification. All results are sample specific.

INVESTIGATION	RESULT VALUE	UNIT	REFERENCE RANGE
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HAEMATOLOGY REPORT

COMPLETE BLOOD COUNT (CBC)

HAEMOGLOBIN (HB)	<u>10.7</u>	gm/dl	13.5 - 17.5
TOTAL COUNT	<u>11560</u>	Cells/cumm	4000 - 11000
NEUTROPHILS	<u>83.9</u>	%	35 - 66
LYMPHOCYTES	<u>9.2</u>	%	24 - 44
MONOCYTES	<u>6.4</u>	%	3 - 6
EOSINOPHILS	0.3	%	0 - 8
BASOPHILS	0.2	%	0 - 1
PLATELET COUNT	1.60	Lakhs/cumm	1.5 - 4.5
RBC COUNT	4.13	mill/cumm	4.5 - 5.9
PACKED CELL VOLUME(PCV)	<u>32.4</u>	%	37 - 53
MCV	<u>78.5</u>	fl	80 - 100
MCH	25.9	pg	25 - 35
MCHC	33.0	g/dl	32 - 36
RDW-CV	13.2	%	11.5 - 14
MEAN PLATELET VOLUME(MPV)	10.6	fL	6.5 - 12

End of Report

Please correlate laboratory results clinically. Contact laboratory for any clarification. All results are sample specific.

Clinical data: Uncontrolled T2 DM

C. T. KUB (PLAIN)

Protocol :

Plain axial CT scan of the KUB region.

Reformatted coronal and sagittal MPR images.

The Study Reveals

• Kidneys:

- Bilateral perinephric fat plane stranding noted.
- Both kidneys are mildly bulky in size
 - Right kidney measures ~ 12.3 x 6.4 x 6.5 cm. No evidence of hydroureteronephrosis. No calculus seen.
 - Left kidney measures ~ 12.0 x 6.0 x 6.1 cm. No evidence of hydroureteronephrosis. Tiny 1-2 mm calculus noted in the lower pole (496 HU)
- Mild bilateral anterior pararenal (Gerota's), posterior pararenal (Zuckerkandl's) and lateral conal fascial thickening noted.
- No demonstrable calculus in the region of ureters on either side.
- Urinary bladder is partially distended. Foley's bulb noted in situ.
- The visualized portions of the spleen appears normal.
- Liver is enlarged in size (20.0 cm).
- Conglomerated calculus noted in the neck of gall bladder, measuring ~ 1.4 cm.
- Survey of pancreas the of the covered bowel loops grossly appears unremarkable.
- Minimal ascites noted.
- Bilateral mild pleural effusion with underlying subsegmental lung consolidation noted.

IMPRESSION

- ✧ Both kidneys are mildly bulky in size with perinephric fat plane stranding and Mild bilateral anterior pararenal (Gerota's), posterior pararenal (Zuckerkandl's) and lateral conal fascial thickening

→ Features likely suggestive of acute pyelonephritis. Suggested contrast study for further evaluation.

- ✧ Tiny left renal calyceal calculus.
- ✧ Hepatomegaly
- ✧ Cholelithiasis without cholecystitis.
- ✧ Minimal ascites.
- ✧ Bilateral mild pleural effusion with underlying subsegmental lung consolidation.

SAMPLE : URINE FOR CULTURE AND SENSITIVITY-

REPORT : NO SIGNIFICANT BACTERIURIA.

Colony count : Less Than 10,000 CFU/ML

—End of Report—

DRUG CHART (1)

DRUGS IN ONLY CAPITAL LETTERS & SIGNED BY DOCTOR

Allergies (if any)		Not Known						
Diagnosis :		T2DM. AFI						
Weight :		71 Kg						
Regular Prescription		D1	D2	D3	D4	D5	D6	D7
Drug		INS						
MEROMAC.		28/2/26	1/3/26	2/3/26				
		9 AM	6 AM	6 AM				
Dose	1 gm.							
Route	IV							
Freq	1-1-1	1 AM	2 PM	2 PM				
Date	28/2/26							
Doctor's signature		-	10 PM					
Drug		INS						
PAN		28/2/26	1/3/26	2/3/26				
		6 AM	6 AM	6 AM				
Dose	40 mg							
Route	IV							
Freq	1-0-0							
Date	28/2/26							
Doctor's signature								
Drug		INS						
EMESET		28/2/26	1/3/26	2/3/26				
		2 PM	6 AM	6 AM				
Dose	4 mg							
Route	IV							
Freq	1-1-1	10 PM	2 PM	2 PM				
Date	28/2/26							
Doctor's signature		-	10 PM					

DRUG CHART (1)

DRUGS IN ONLY CAPITAL LETTERS & SIGNED BY DOCTOR

Allergies (if any)		Not known						
Diagnosis :		DK1A T2DM.						
Weight :		71kg						
Regular Prescription		D1	D2	D3	D4	D5	D6	D7
Drug	INS	28/2/26	1/3/26	2/3/26				
	Sumol	4M	3:30AM	2AM				
Dose	19m							
Route	IV							
Freq	SOS	11:30PM GUSP						
Date	28/2/26							
Doctor's signature								
Drug	INS	28/2/26	1/3/26					
	HAPPYNERVE PLUS.	6 AM	2M					
Dose	1amp							
Route	IV							
Freq	0-1-0							
Date	28/2/26							
Doctor's signature								
Drug	INS	28/2/26	1/3/26	2/3/26				
	H. ACTRAPID.	8 AM	9:30 AM	10 AM				
Dose	S/S	10 units	12 units					
Route	SLC							
Freq	1-1-1	2M	2M					
Date	28/2/26	5 PM	5 PM					
Doctor's signature		11 PM	9 PM					

DRUG CHART (1)

DRUGS IN ONLY CAPITAL LETTERS & SIGNED BY DOCTOR

Allergies (if any)		Not Known						
Diagnosis :		DRA. AFI						
Weight :		71kg						
Regular Prescription		D1	D2	D3	D4	D5	D6	D7
Drug	INS LANTUS	28/2/26 1pm	1/3/26 2pm	2/3/26 2pm				
Dose	10 U							
Route	slc							
Freq	OD.							
Date	28/2/26							
Doctor's signature								
Drug	TAB DOLO	28/2/26 9 AM SPR	1/3/26 10 AM	2/3/26 10 AM				
Dose	650mg							
Route	P/O							
Freq	1-1-1							
Date	28/2/26							
Doctor's signature								
Drug	SEP Gel	1/3/26 2pm	2/3/26 9 AM					
Dose	L/A							
Route	P/A							
Freq	1-1-1							
Date	28/2/26							
Doctor's signature								

DRUG CHART (2)

DRUGS IN ONLY CAPITAL LETTERS & SIGNED BY DOCTOR

Weight:		71kg						NEBULISATION	
Drug & Dosage									
Date	Doctor's signature								
Drug & Dosage									
Date	Doctor's signature								
Drug & Dosage									
Date	Doctor's signature								

ONCE ONLY AND PREMEDICATION DRUGS :

Date Time	Drug (Approved Name)	Dose	Route	Min Hr	Dr.'s Sign.	Given by

INTRAVENOUS FLUIDS AND CONTINUOUS INFUSIONS :

Date Time	IV Fluid Type	ML/Hr	Stop order
28/2/26	NSLR @ 150ml/hr		
8 1pm	orally soft diet		

CASE RECORD / ADMISSION RECORD (1)

Informants:

Referred by : Dr.

Chief Complaints :

Qo Fever
Generalized weakness Since 3 days
Myalgia. ⊕

History of Present Illness :

Patient on his regular medication
Gradually Qo Fever, Myalgia, Generalized weakness
for further Dr came to our hospital

Past History / Drug History:

KHCO T2DM (on Ayurvedic Medication)
HbA1c - 13.9% (outside report)

Allergic History:

Not known

CASE RECORD / ADMISSION RECORD (2)

Examination: olt Conscious, oriented

General Physical: well Built

PR: 116	b/m	BP: 87/50	mm Hg	RR: 16	/m	SPO2: 99%	%
Wt:	kg	GRBS: 443	mg/dL	Temp: 98.3	°F		

Pallor ☐ Icterus ☐ clubbing ☐ kolinychia ☐ Lymphadenopathy ☐ Edema ☐

Systemic examination:

CNS: 1) GCS : M6/V5/E4 6 / 5 / 4 / 15

2) Pupils: equal

3) Tone:

4) Plantar

5) Power:

6) Meningeal Signs

7) DTR:

8) Involuntary movements:

CVS : S1 ☒ S2 ☒ Murmurs ☐ Grade I II III IV Gallop ☐

RS : Efforts : Normal ☒ Poor ☐ Increased ☐ Acidotic ☐

Retractions: Subcostal ☐ Intercostal ☐ Sublotic ☐

Air entry : B/L Equal ☒ Decreased Air entry : Left ☐ Right ☐

Chest sounds: Ronchi ☒ Crepts ☒ Stridor ☐

PA : Soft ☒ Distention ☐ BS ☐ Liver span _____ cms, Spleen span _____ cms, AG _____ cms

**CASE RECORD
/ ADMISSION RECORD (3)**

Examinations:

Investigations:

CBC, Sr Creatinine, C-peptide, HbA1c
ABG, Urine @

Diagnosis(Provisional):

7AR2

Uncontrolled T2DM

Final Diagnosis:

- ? synovitis
- cholelithiasis w/throw cholecystitis

Resident Dc

Name



Consultant Signature

CONSULTATION SHEET

Known Drug Allergies

Not Known

Date :

27/2/26
11:30 Am

CLSB Dr.

Care details reviewed

care of uncontrolled T2 DM.

AKI

WC : fair

O/E

BP - 110/70 mmHg

PR - 117 bpm

SPO₂ - 94%

PA - soft

CNS - Conscious,
oriented

CVS - S1 S2 ⊕

RS - B/L AET ⊕

Advt use abd & pelvic - stat

CRBS 2nd hourly

→ CBL

→ S. Nat

→ Urine routine

S. Nat

→ Shift to ward by evening

@ 3Pm

CONSULTATION SHEET

Known Drug Allergies

Not Known

Date :

27/02/26

2D Echo

Sinustachycardia @ HR - 149 bpm

Normal LV systolic function

No definite RONA

LVEF - 60%

AR - Trivial, NO AS

MR - Trivial

TR - mild

NO PAM (RVSP - 28 mmHg)

LC 12/6 mm normal collapsing

No dx/pericardial effusion

D. B. S. O. U. R. A. V.

CONSULTATION SHEET

Known Drug Allergies

— Not known

Date :

27/02/26
15:20pm

C/S/B Dr.

Case details reviewed

AC. Status Quo

USG slo. pyelonephritis?

BP: 96/60 mmHg

HR: 116 bpm

SpO₂: 99-100% O₂

CS: conscious

US: SIS ⊕

RIS: BIC NBI ⊕

PIA: soft, diffuse tenderness

Ad

- CT KUB

- urologist opinion

- CBI, SN sent TIM

- Rx at per chart

CONSULTATION SHEET

Known Drug Allergies

Not Known

Date :

23/2/26
11:40 AM

CLSB DR.

care de tail reviewed
AC: fair

O/E

BP - 110/70 mmHg

PR - 114 bpm

SpO₂ - 96%

PA - soft

CNS - Conscious,
Oriented.

CVS - S₁S₂ ⊕

RS - B/LAE ⊕

Adv:-

→ Urologist opinion

→ CBC
S. creat / Tm.

→ Shift to ward.

→ Rx as per chart

CONSULTATION SHEET

Known Drug Allergies

Not known

Date :

28/2/2026
11:45 AM

S/B Dr

Thanks for referral

History noted

Reports noted

CT-KUB (Films) noted

Ass

→ Reassured

→ continue same Treatment

CONSULTATION SHEET

Known Drug Allergies

Date

28/2/26
5:34 pm

— NO —
C/S/B Dr.

Case reviewed

Case of AFI, DKA

Uncontrolled T₂DM.
B/L Pyelonephritis

C/O Dryness of mouth
- febrile @ 4pm
102.2°F

O/E:

Bp - 130/80 mmHg

HR - 135 bpm

Spo₂ - 91% @ RA

CNS - Conscious, oriented

CVS - S₁, S₂ (+)

RS - B/L Ate (+)

p/A - Soft

Advice:

CBC

Sr. Creat

GPB - 297

T/M

- FC remove Stm

- 8th body GPBs monitor

CONSULTATION SHEET

Known Drug Allergies

— No —

Date :

1/3/22
11:30 AM

Ch (By Dr

— Case reviewed

— pt is stable.

— Episode of shills
yesterday night.

— No fever spike etc

— Symptomatically
better now

BB-110/30mmHg
PR-80bpm
SpO₂-98.6
@RA

Admit

— C S T

CONSULTATION SHEET

Known Drug Allergies

— NO —

Date :

02/03/2026

C/S/B Dr

11:25 AM

Case reviewed

Case Of AFI, DKA

Uncontrolled T₂DM

B/L Polyneuropathy

O/E :-

BP: 170/80mm Hg

SPO₂: 97%

HR: 110 b/m

CNS: - Conscious
oriented

CVS: - S₁, S₂ (+)

Advice :-

→ Ortho opinion

→ Discharge on Request (Evening)

→ Diet counselling

CONSULTATION SHEET

Known Drug Allergies

Date :

2/3/26
11:56 AM

c/s/B Dr.

- NO —
- Thanks for reference
 - History noted
 - C/o pain @ hip region

2/6

- Tenderness @
- Range of movement painful & restricted
- H/o @ tibia operated 3 yrs back

Adv:

→ X-ray pelvis - B/L hips - AP view

X-ray @ # noted well

Rx ? synovitis

T. Ultracet 1-2 x 5 day

Continue
Education

~~T. Ultracet 1-2 x 5 day~~

T. Flashine
907010



sarji
Trusted empathy in healthcare

SUPER
SPECIALITY
HOSPITAL



CONSULTATION SHEET

Known Drug Allergies

Date :

2/3/26
1 pm

— NO —

Cs/B Dietician

Thanks for reference

Case reviewed

do - AFI / DKA .

BL pyelonephritis.

Uncontrolled T2DM on Ins

Diet counselling done & diet chart issued

Adv

- Strict diet modification

- Allow low GI foods

Include fiber rich foods

MONITORING CHART

TIME	Tmpr °F	HR per min	RR b/min	SPO2 %	BP mm/Hg	GRBS mg/dl	O2 min/ltr	Staff Name & ID
28/dec								
4pm	102.2°f	135b/m	20b/m	87%	130/70mmHg		Room	
7pm	98.6°f	120b/m	21b/m	86%	130/80mmHg		Room	
9pm	98.4°f	110b/m	20b/m	90b/m	120/80mmHg		Room	
11pm	102.2°f	120b/m	21b/m	89%	130/80mmHg		Room	
1/3/26								
12AM	98.6°f	110b/m	20b/m	89%	120/80mmHg		Room	
2AM	98.4°f	112b/m	21b/m	90%	110/70mmHg		Room	
7AM	98.6°f	110b/m	20b/m	89%	120/80mmHg		Room	
9am	97.4°f	100b/m	20b/m	93%	110/70mmHg		Room	
12pm	97.4°f	98b/m	20b/m	95%	110/60mmHg		Room	
2pm	97.2°f	100b/m	20b/m	96%	110/70mmHg		Room	
4pm	98.2°f	100b/m	20b/m	97%	120/80mmHg		Room	
6:43pm	99.1°f	110b/m	22b/m	98%	130/60mmHg		Room	
9pm	98.4°f	110b/m	21b/m	96%	130/80mmHg		Room	
2/3/26								
12pm	98.6°f	111b/m	20b/m	98%	120/80mmHg		Room	
2pm	102°f	100b/m	21b/m	93%	110/80mmHg		Room	
7AM	98.6°f	110b/m	20b/m	98%	120/80mmHg		Room	
10AM	97.6°f	100b/m	20b/m	94%	110/80mmHg		Room	
12pm	98.2°f	110b/m	20b/m	96%	120/60mmHg		Room	
2pm	97.2°f	100b/m	20b/m	97%	110/70mmHg		Room	

CERTIFIED

GRAPHIC(TPR)CHART

DATE	28/12/25	1/3/26	2/3/26					
NO. OF DAYS	01	02	03					
DAYS POST OP								
TIME								
PULSE								
TEMP								
210								105.8
200								104
190								102.2
180								100.4
170								98.6
160								96.8
150								95
140								
130								
120								
110								
100								
90								
80								
70								
60								
50								
40								
DATE	28/12/25	1/3/26	2/3/26					
URINE	flc	selte	selte					
BATH	com	glen	glen					
WEIGHT	-		file					
DIET	@	(n)	(n)					
SIGNATURE								

INTAKE/OUTPUT CHART

[illegible]

INTAKE/OUTPUT CHART							
TIME	INTAKE				OUTPUT		
	Parenteral	Qunatity	Oral/RT	Quantity	Self void urine Drainage	Vomitur / Bowels	Ryle's Tube Aspiration
1/3/26 9 AM			H ₂ O	200ml	150ml		
10 AM							
11 AM							
12 PM					300ml		
1 PM			H ₂ O	100ml			
2 PM							
3 PM			H ₂ O	150ml	200ml		
4 PM							
5 PM							
6 PM	Pnj	100ml	H ₂ O	50ml	200ml		
7 PM							
8 PM							
9 PM							
10 PM							
11 PM			H ₂ O	100ml		and take - 750ml	
2/3/26 12 AM					200ml	output - 1250ml	
1 AM							
2 AM						(quy)	
3 AM							
4 AM			H ₂ O	50ml			
5 AM							
6 AM					200ml		
7 AM							
8 AM							

INTAKE/OUTPUT CHART

[illegible]

NURSING DOCUMENTATION

(From.....8pm.....to.....8AM.....)

Date: 26/2/26.

8pm Patient received from casualty to HDU

- Patient vital signs connected.
- Patient hand over taken from casualty staff to HDU staff.

8:30pm → Patient Side Rounds done

→ Patient conscious and oriented.

→ Patient IV cannula present.

→ Patient orally left diet present.

→ Patient self voided present.

→ Patient IVF NS/RL @ 150ml/hr

if GRBS is less than 200 mg/dl

DNS 150ml/hr.

→ Patient hourly GRBS present.

→ Patient vital signs monitored

Temp :- 98°F

Bp :- 100/60 mmHg.

HR :- 99 bt/min

RR :- 26 br/min

SpO₂ :- 98%

@ 9pm Patient had hypotension informed to

and advice

IVF NS 200ml bolus given as per advice

NURSING DOCUMENTATION

(From.....8pm.....to.....8AM.....)

Date: 26/2/26

→ patient had a chills informed to

advice 8mg tramadol 2ml + 3ml NS = 5ml. 3ml IV & 2ml in 500ml NS as per advice given.

→ Patient had a sudden desaturation

→ Patient O₂ mask connected.

@ 10pm Patient IV cannulization done

→ Patient All IV medication given.

→ Patient Inf - Human Actrapid. 2ml/hr on flow.

→ Patient hourly GRBS checked and informed to

11pm → Patient had a hypotension informed to

as advice. 500ml bolus as per advice given

@ 1pm → Patient vital signs monitored and recorded.

@ 2pm in advice Foley's catheterization as per advice as per advice done.

@ 4AM Patient Intake output done

NURSING DOCUMENTATION

(From.....2pm.....to.....5AM.....)

Date: 26/2/26

@5AM Patient Bud making done, Bed
bath given, comfortable
position given.

@6AM Patient IV medication Inj-PAN/way
Inj-Emesetmg given as per
drug chart

@7AM Patient hourly GRBS done

→ ~~patient~~

→

advice Inj-
Sodabycarbonate 25ml in NS 2 cycles
as per advice given.

@8AM Patient Hand over given to
morning duty staff.

NURSING DOCUMENTATION

(From.....8Am.....to.....2pm.....)

Date: 27/2/26

8:30 Am

⇒ patient charge handover taken from night duty staff to morning duty staff

→ patient side rounds done

→ patient ID Band is present

→ patient IV cannula present both hand cannula 20h cannula left hand 18h cannula RT hand

→ patient general assessment done

→ patient F/C present 16 number

→ patient vital sign checked & Record

BP - 108/64 mm/hg

SpO₂ - 96%

HR - 114 b/m

RR - 28 b/m

→ patient I/O monitor

⇒ patient comfortable position

⇒ patient Seen By Dr.

Sing Aelu TO.

By Laner wuni sk.

⇒ patient okay say dit

⇒ patient IUP fluids 100ml per hour on going

NURSING DOCUMENTATION

(From.....to.....)

Date: 27/2/20

11:00am => patient seen
sampled in Adv'D

=> CBC,

② Sn. cultures

③ Sn. Nat

④ Urinal results

⑤ Use Abdominal pelvis

=> plan sh to ward.

=> patient vital signs checked
and recorded

12pm => patient URBS is checked
1pm and recorded 309 Dypom
in

2pm => patient Echo reported.
patient drug hand over
given to Monitor duty
Staff to Evening duty
Staff

NURSING DOCUMENTATION

(From.....2pm.....to.....3pm.....)

Date: 27/2/2020

2pm ⇒ Patient hand over taken from Morning duty staff Nurse
 ⇒ Patient General Nursing Assessment done & Recorded.
 ⇒ Vitals checked & Recorded.
 HR - 142b/min Bp - 120/65mmHg
 RR - 28b/min T - 98°F
 SpO₂ - 98% on 2L O₂ NP.
 ⇒ Patient Conscious & Oriented.
 ⇒ Patient's both hand side IV Canula present IVF 100ml/hr NS on flow.

2:30pm ⇒ Patient's T - 102°F Infund to Dr. Sign & Iny PCT 1gm, Iny given.

3:55pm ⇒ Ure, Abdomen & pelvis done by Doctor Report Awaited

3pm Urine Routine & CBC, Sr. creatine and Sr. Sodium Send to Lab Report Awaited

3:10pm ⇒ Dr. Adric to send Blood and Urine Culture, due to Fever Spike.

4:40pm CBC, Sodium, Sr. creatine Report Infund to Dr

NURSING DOCUMENTATION

(From.....2pm.....to.....8pm.....)

Date: 27/2/2026

4:40pm ⇒ Bld Cls and Urn cls Send to lab Report. done

6pm ⇒ Dr. Sin Rounds

Done & Advis'

* CT KUB

* Urologist opinion

* CBC. Sr. create T/m

* Rx as per chart.

7pm ⇒ GRBS checked & Recorded
Infused to Dr Shire sin

LIE
Gpm ⇒

Inf meromac 4gm (ATD)

Tr gins as per chod.

⇒ CT KUB. Bill paid.
CT Scan done.

⇒ Patient hand on gins to
Night duty staff Reu

